

Understanding the experiences related to mental health amongst 16 – 24 year olds who feel under represented; a narrative inquiry study

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Abstract [336 words]

Background: The majority of mental illnesses emerge before the age of 25. It is well established that those who are most vulnerable and most in need of mental health support are least likely to access services or take part in research.

Aims: Adopting an intersectional lens, this research aims to elicit the experience regarding mental health of young people who feel under represented

Method; People aged 16 – 24, living in England, who were currently experiencing or had experienced poor mental health in the preceding year were recruited from both NHS waiting lists (in Oxford, Sheffield, Cornwall and Liverpool) and nationally on social media and via charities. 248 people completed an online questionnaire, of which 117 took part in an initial screening call and 81 were interviewed. Unstructured interviews that adopted a narrative inquiry approach were conducted either in person or on online, and audio recorded then transcribed verbatim. Data were analysed using a combination of thematic and framework analysis. Young people with relevant lived experience consulted on the implementation of the research and recruitment, and peer researchers contributed data collection and data analysis.

Results: Participants engaged with unstructured narrative inquiry interviews, often anchoring their experiences either in their schooling, or in a series of adverse childhood events. Three overarching themes were extracted from the data; the impact of connection, (in)validation, proactively coping and preventive opportunities. These themes highlighted the variation in health literacy and how mental health experiences are understood, as well as whether existing services were experienced as accessible and helpful. Participants were proactive with adopting coping mechanisms in the absence of support and identified times of transition as providing an opportunity for improved support.

Discussion: There are implications for policy and practice in how mental health is talked about and recognised amongst young people. It is important that young people are empowered to talk about their experiences, and that attempts to seek help or access care are not met with invalidation. Times of transition (particularly around schooling) are opportunities where support could be better offered in a preventative way.

Strengths

- Large diverse set of lived experience data
- Youth-led involvement strategy for implementing the research programme
- Inclusive and intersectional approach
- Open methodology that enabled participants to set agenda

Limitations

- Ability to represent large diverse data set within existing analytic frameworks
- Unstructured narrative interview methods potentially challenging for some neurodivergent people and those with communication needs.

Introduction [572 words]

The World Health Organisation report that globally around one in seven young people (aged 10 – 19) experience a mental health disorder, with suicide being the third leading cause of death in adolescents (aged 15 – 29)[1]. Poor mental health has been the leading health and social issue impacting young people, with the mental health of emerging adults declining steadily over the last two decades[2]. Approximately three-quarters (63 – 75%) of mental disorders emerge before the age of 25, yet young people are the least likely to receive appropriate care[3]. Common mental health problems in young people, such as depression and anxiety have long- term implications on mental and physical health, social support, relationships, levels of employment, as well as health risk behaviours including self-harm[4]. There are significant direct and indirect costs over the life-course[3].

People from marginalised groups experience higher proportions of mental health difficulties[5]. There are many forms of marginalisation, by gender, by ethnicity, by neurodivergence or by socioeconomic status are a few that have been frequently explored in existing literature[6]. There is ample evidence that escalation of poor mental health is particularly evident amongst girls [5, 7, 8], and that people from minoritised ethnic backgrounds experience specific challenges in accessing mental health support[9]. Many autistic young people lack suitable psycho-social support which compounds mental health inequalities, yet there is little research into their mental health needs or into how best to engage them in research as equal partners[10]. Much of the existing evidence base relating to autism has been derived from research that considers professionals and parents perspectives rather than asking autistic young people themselves[11]. This is a form of epistemic injustice, which occurs when existing prejudice causes the knowledge or testimony of an individual to be given less credibility [12], and has historically impacted the ability of marginalised groups to contribute to what is known about their experiences.

Within these different forms of marginalisation, it is important to adopt a lens of intersectionality, and recognise that some people may experience multiple forms of marginalisation, for example an autistic black, female. The approach to exploring these nuances should recognise the context that creates a system by which certain types of people are at a disadvantage [6]. For example a qualitative study in Scotland exploring young women's (aged 16 – 25) experiences of mental health inequalities found that structural disadvantages such as racism intersect with gender and age[13]. An approach that focuses solely on one form of marginalisation or intersection, risks further excluding those who experience multiple forms of marginalisation and thus perpetuating existing inequalities[14].

Research that aims to better understand the experiences of those who are not well represented in the existing evidence base on young people's mental health needs to be co-curated with young people who experience multiple forms of marginalisation to ensure that it is truly accessible to the target populations. Whilst there is evidence for effective interventions to address common mental disorders such as anxiety and depression amongst young people[15], the mechanisms for accessing these interventions are not adequate[16], and often subject to budget cuts[17].

This research aims to create an opportunity for young people to share their narrative and mental health experiences without a pre-defined structure in a way that accommodates individual needs and allows for participants to share what feels important to them. This information allows for the research team to better understand their experiences regarding mental health and inform future policy and practice around mental health interventions.

Methods [1,796 words]

Design

Narrative inquiry with an intersectionality lens was adopted to elucidate the experiences of people aged 16 – 24 who feel under represented living in England, around mental health.

NHS Research Ethics Committee and Health Research Authority (24/SC/0083) approvals were granted on 26th March 2024. The materials that were submitted to the ethics committee were iterated with and reviewed by the Origin Research Advisory Group (ORAG) comprised of people aged 16 -24 with relevant lived experience. The protocol for this work has been published[18].

Participants

The literature referenced in the introduction of this paper often uses the term ‘marginalised’. It was important to work with young people to consider how to operationalise this term in a meaningful way that did not exclude or alienate the populations who we wanted to include in this work.

The term ‘underrepresented’ in this title of the broader research programme, this refers to people who are underrepresented regarding the existing mental health evidence base. This was discussed in depth owing to differences in interpretation of the term underrepresentation. For young people under represented often related to not feeling like they had access to anyone who was like them regarding a particular experience. This feeling of being underrepresented and not having access to anyone ‘like them’, was felt by the young people working on the research programme to be an adequate way to reach young people who are marginalised with regard to research without reducing them to a set of characteristics.

The inclusion criteria for this work was;

- Aged between 16 – 24
- Living in England
- Currently experiencing or have experienced (over the past year) a mental health problem.

Young people with relevant lived experience were consulted throughout the recruitment process on a weekly basis to consider which underrepresented groups were being included and whether specific groups should be targeted whilst recruiting.

Recruitment

There were two ways in which people were recruited to take part;

1. Online – organisations and charities nationally were targeted as well as those located in Sheffield, Oxfordshire, Cornwall and Devon, and Liverpool with high levels of engagement from under-represented populations. Youth informed methods of reaching underrepresented young people online were used, such as adverts on social media and the mailing lists for charities like YoungMinds.
2. NHS Waitlists - CRN teams contacted people on waitlists who had not yet engaged in an NHS service, or have an allocated care team. In each location relevant staff from the research department sent out the advertising materials and links to potential participants alongside contact details for the central research team in case potential participants had questions.

Of the 273 who completed the screening questionnaire, 84 were recruited via NHS trusts (in Oxford, Sheffield, Liverpool and Cornwall), and 161 via community organisations and social media. As published [18] the research team endeavoured to also recruit from Blackpool, but connections with community organisations there did not come to fruition. When potential participants accessed the study online, they were presented with an information sheet and given the opportunity to self-identify initial eligibility (i.e. age, location and having experienced mental health difficulties in the preceding

year) and to consent to participation. Those selected via purposeful sampling were sent the information sheet via their preferred contact method and contacted by a member of the research team to arrange an initial conversation.

Data collection

The approach to facilitating participants to share their experiences was developed in a series of workshops and training sessions that were held with a group of 10 -12 people aged 16 – 24 working on ORIGIN. Throughout this process the young people we were working with recognised the importance of an inclusive, reflexive space but did not want to feel pressured to share personal feeling and reflections. Throughout the interview phase monthly sessions were held that were alternated between being online and in person where transcripts and processes were reviewed. This structure also allowed space for people to reflect on and share their own experiences and challenges of working on the programme of research.

The initial recruitment target for interview completion as published [18] was 100. Between May 2024 and April 2025, 283 responses to the screening questionnaire were received of which 218 were complete and valid. Initially every participant who was eligible was invited to an interview, after 20 interviews had been conducted the team (including young people) regularly reviewed the demographics of both those being interviewed and those completing the questionnaire to allow for a more purposeful approach.

Screening calls provided an opportunity for potential participants to ask questions and for the research team to ascertain their availability, as well as talk through their preferences to ensure that that interview process was accessible and comfortable for participants to share their experiences. This included the modality of the interview (in person or online), potential location requirements, and other things such as whether they would like to bring anything (fidget toys) and what their preferred method of communication was. 165 were invited to a screening call and 117 attended a screening call, from which 73 interviews were completed, an additional 8 interviews were conducted as a result of targeted recruitment in Cornwall to better understand what adjustments were needed to include neurodivergent populations with a high level of need. Whilst this was short of the initial recruitment target of 100 [18], from discussions with the young people, the research team determined that the sample was sufficiently diverse; the only group identified as being important by the young people who were invited to interview and not represented in the final sample were young parents.

Whilst narrative inquiry often uses a single question [19], we developed an opening statement to provide more context. This approach was adapted from existing research [20]. Later during one of the reflective sessions once a few interviews had been conducted a closing statement was developed to ensure standardisation across the team. All online interviews were conducted on Microsoft teams, in person interviews were conducted in a variety of spaces from outdoor parks to community centres and museums. Once interviews had been completed, participants were asked if they would like a member of the research team to check in with them a few days or a week after and offered further signposting. Participants were sent a love2shop voucher worth £40 after being interviewed and where requested a bank transfer was facilitated.

Audio recordings of the interviews were transcribed automatically using Microsoft Teams, then verified by the researcher who led the interview and anonymised. These transcripts were then sent to the peer researcher, or where there was not a peer researcher a second listener (who was a young person who was involved in the research but not as a participant) to verify the transcription and add any further reflections.

Data Analysis

An accessible, rigorous and thematic approach was taken to exploring the transcripts. It would be usual in narrative inquiry approaches to adopt a series of case studies and case wise comparisons[19], but this was not possible owing to the volume of data. Training on thematic analysis was provided to the

peer researchers. Everyone involved (both peer researchers and members of the research team) coded the same transcript and then came together to discuss differences and challenges. This session was conducted in person to allow the team to also consider the individual impact of reading such stories, that often included descriptions of adverse childhood events such as abuse, racism or self-harm. Microsoft Excel was used in lieu of qualitative software to ensure that the analytic process was accessible in a timely manner to the peer researchers. Many of the young people coding the transcripts had also been involved in either conducting the interviews or data verification to familiarise themselves with the data set.

Once the transcripts had been coded, two researchers independently summarised the codes for 10 different transcripts and then came together to combine them. These summaries were shared with the broader team. A meeting was held in person with 5 of the young people to review all of the theme summaries in detail and ensure that none of their coding had been misinterpreted or lost in the process of summarising a large volume of data.

Reflexivity and positionality

During the data collection and analysis phase monthly sessions were held with everyone who was working directly on these tasks (including young people who were working as peer researchers and youth co-ordinators). These were led by RM, who is a woman of colour with more than 10 years experience of working in participatory inequalities research. Within these sessions the team openly discussed and addressed power dynamics as well as considered the bi directional relationship with the data. Young people with relevant lived experience aided the development of the opening statement for the interviews which impacted the information shared by participants, and co-developed the coding framework. The voices of young people with relevant lived experience were foregrounded at every stage of implementing this research.

Accessibility adjustments

In Cornwall a collaboration with the CIDER centre facilitated the participation of young autistic people with relatively higher needs where about 5% of people diagnosed with autism are in this category. At greater risk of isolation from medical services generally and health research in particular, the DSM-V diagnostic manual is often used to define this target group broadly at level 2 (out of 3), that is, requiring substantial support in engaging with their day-to-day life. It is important to acknowledge that the impact on daily life of autism can vary widely between individuals and even within an individual depending on life stage, environment and context. Amendments were made to the information sheet informed by peer researchers to include having an audio recording of the text and language adjustments. Recruitment was mainly through NHS pathways to ensure that a range of individuals in the target group were included. The data from autistic recruitment in Cornwall (n=8) was analysed in a separate but similar process. Analysis was undertaken by a team of two, a member of the research team and a peer researcher who is an autistic young person. After data familiarisation, both coded the same two transcripts independently, in a broadly inductive coding process. The resulting coding structures were compared and amalgamated jointly as the data was entered into NVivo. The remaining transcripts were then coded in NVivo using the agreed coding structure, which was amended as required. The team of two then met in person to organise the codes into themes. The research team member summarised the entries against individual codes and then summarised the codes into themes. The analysis was collated to a framework matrix in Microsoft Excel which was reviewed and edited by the peer researcher. The result was reviewed by a wider group of two autistic peer researchers, and combined with the analysis from the other interviews.

Results [2,954 words, plus 2,332 words that are quotes]

81 interviews were conducted. 20 of these were online, 61 were in person and 21 were attended by a peer researcher. Interviews lasted between 25 mins and 123 mins, with the average around 60 minutes. There was no difference in the average length between online and in person interviews. Very few participants requested a call back at the end of the interview (n=6) and participants often reported that they enjoyed the interview process and opportunity to tell their whole story, for some it was the first time talking about their mental health. The demographics for the participants who were interviewed were distributed as detailed below, with regard to;

- Age; 16-18 (n=32, 40%), 19 – 21 (n=20, 25%), 22 – 24 (n=29, 35%)
- Gender; Female (n= 47, 58%), Male (n=25, 31%), Non-Binary (n=5, 6%), Other (n=4, 5%)
- Ethnicity; White (n = 55, 67%), Black (n=15, 18%), Asian (n=5, 6%), Arab (n= 1, 1%), Mixed (n=5, 6%)
- Neurodiversity Diagnosis; Yes (n=52, 64%), No (n=29, 36%)

More detailed demographic information was collected as detailed in the protocol[18], and will reported separately to also consider the demographics of those who completed the screening questionnaire. Three themes were extracted from the data which are exemplified with quotes below. For participants who were recruited from Cornwall and identified as having a high level of support needs the participant number is prefixed with the number 2.

Theme (from iterative discussion with young people)	Subthemes from the framework (that was co-developed with young people after all peer researchers and researchers inductively coded one transcript and iterated the framework with a further five transcripts)
The impact of connection	Isolation and connection Family and friends Intimate relationships Sexuality and Gender Mental health experience
(In)validation	Accessing help Masking Self-awareness/Emotions Culture and traditions
Preventive opportunities and proactively coping	Transition Trauma School/College/University/Employment

1. The Impact of Connection

Participants talked about the impact of being able to connect and relate to the people around them. Whether an experience was ‘normal’ or not was appraised based on the experiences of the people around them who they were able to relate to. This meant that experiences were often perceived as ‘normal’ and therefore not warranting help, until the participant was able to relate to someone who had been in a similar situation and had already received help. Connections to other people often facilitated identifying and naming challenges (for mental and physical health and neurodivergence) which in turn enabled participants to access help.

I got to meet different, other people's with the same problems and we talked and shared and had conversations where we talked about ourselves and what um, the way we were helping ourselves, the things you're doing to develop ourselves, the kind of supports you need and the kind of supports that you feel like should be given to us (P123)

Prior to establishing meaningful connections participants shared a number of ways in which they felt isolated, such as physically being alone, feeling alone due to bullying, being misunderstood or unable to communicate with others, as well as self isolating as a coping strategy. Male participants also shared experiences of stigma and the importance of addressing myths that boys cannot struggle with poor mental health.

You get so many people like that. Like, that's all kinds of, like you're a man and you cry and you're man and you cry and all that. I've heard that so many times. Even for my friends, even from people I'm close to. Even for my brothers. And then of course for my dad. And that was like the same like your crying that makes you a bitch (P049)

Adverse childhood experiences relating to family dynamics (including bereavements, parental separation, abuse, unstable sibling relationships and poor parental mental health) often negatively impacted participants ability to connect with others.

At least I'm providing for my siblings and stuff. OK, I've been tough though. Oh my sister should be going to college. And I'm just trying my best you know to support her. Give her the help she need. Errr be the provider that she didn't have throughout her lifetime and all the rest so (P11)

Vulnerability engendered by abusive trauma was a mental health determinate in half the cases in the autistic sample with a high level of need. The trajectory of sexual relationships featured prominently, although also, in some cases, as a positive factor.

And I changed a lot when we were dating and did things that I probably wouldn't have done before we were dating. And, Yeah, it's also hard because I get blamed for all of it or he didn't see the problem he'd done, he just saw what I did. ... And then I started to feel really kind of guilty and bad. Like, maybe it was my fault. And I started getting really like upset and depressed throughout the relationship. (2P04)

Supportive friendships were integral to coping with poor mental health and reinforced a sense of feeling connected to others, though in some cases participants described that the presence of similarities created an echo chamber that was difficult to escape. For some participants friendships were a source of anxiety, particularly in school environments. This was particularly evident amongst participants who declared a form of neurodiversity.

I think a lot of it is like the friends I made at uni as well. Like, we're all kind of, like mentally ill, you know, have issues we all like talk quite freely. So I was quite lucky in that respect. But like, that probably did have quite an effect. I've maybe also just because. Like we talked about mental health very freely, like on Twitter. Because obviously, like I said, everyone was just freely saying, like going to try to kill myself tonight. And it's like, oh, OK. (P026)

Some participants shared that intimate relationships had negatively impacted their mental health, with the breakdown of a relationship often preceding an episode of poor mental health. For others romantic relationships were a source of continuity and support through transitions or advocating for them in different settings.

I don't know why I stayed with him for so long, but I never gave in to any of that. But things did turn quite nasty. But like, it wasn't always nasty. Sometimes the relationship was nice, and I think stemming from feeling like when I was younger that I would never find love and acceptance, I, I accepted the minute that somebody showed a bit of attention towards me, I leapt at it and that was probably why I got into that relationship and why I stayed for so long (P097)

Lack of representation or acknowledgement around diverse sexualities led to discrimination, isolation and confusion. Challenges were often shared where an individuals' experiences of sexuality did not align with the expectations of those around them, which often led to seeking connection in online spaces.

I think it was challenging in some aspects as well because you've got the people's whole like self-identity thing at that point as well. So I'm not straight and my parents are very bigoted people in general, not just towards like LGBT people, but everybody. And so that makes it quite difficult. (P056)

For autistic participants with a high level of need, lack of awareness of autism among family, peers, educational environments and even health settings is a major driver of anxiety, stress and negative encounters with a behavioural model of compliance and punishment from primary school onwards. In contrast, autistic participants themselves had high levels of self-awareness about autistic traits and its nuances including co-occurring conditions and traits such as sensory sensitivity, social anxiety, special interests and the potential for overload leading on occasion to meltdowns.

I think a lot of the stuff that I would get in trouble for was tied to me being autistic. I couldn't deal with change so I would have like meltdowns and then I'd get in a lot of trouble and I'd get really punished for that. And then I would have more anxiety so then my - like I'd struggle more with my autism's like sensory problems and struggling with talking with people. And then I'd get in more trouble and it just kept building as a loop. (2P06)

Diagnosis of autism is a complex issue, characterized by delay, lack of awareness, gender bias and reactive rather than proactive stances. Participants recruited with a high level of need noted they often needed external help to identify potential autism in themselves. They valued diagnosis more for the effect on others than the self but acknowledged the retrospective clarity on challenges from their earlier lives.

It makes sense now. Like a lot of things that have happened with me, they make sense now, like somewhat sense, ever since I got my diagnosis. ... It makes it easier to identify what could be part of my autism and what isn't. So ... It's easier, like in a way, yeah, to deal with it sometimes. But not all the time. (2P05)

2. (in)validation

There were a number of different care pathways that participants had experienced; school/university, hospital (being in hospital, eating disorder units, psychiatric wards), CAMHS and mental health services, helplines, care co-ordination, psychiatrists and psychologists, other health professionals (such as GPs, eating disorder specialists, dieticians), social workers, and counsellors. The majority of services that participants described were not experienced as helpful, participants often shared interactions that made them feel invalidated and a lack of continuity of care. Gaining acknowledgement in school environments for the impact of poor mental health or neurodivergence on attainment in the absence of reasonable adjustments was very challenging.

I personally hate CAMHS. I hope I never have to go through CAMHS again because, umm, I just feel like it it doesn't benefit me in any way and it actually just made me feel a lot worse, better off, umm but yeah. (P064)

Parental beliefs around mental health often impacted whether participants were able to access help, both positively (taking them to the GP or paying for private care) and negatively (not acknowledging poor mental health).

Trying like I've never spoken to a counsellor before. I've never. You know, there's a need for denying a lot. I've got such a way with avoiding. Because if I don't acknowledge it, then it's not that. And often I was. I was just like my parents already told me. You know. Nobody says anything about. Is everything OK? You know when you say how are you? It's more like how are you doing at school, you know? there's a level of avoidance in that sort of thing. So as you're never really naming the emotion that you have (P034)

If they had the resources available some participants sought support privately owing to NHS waitlists or poor interactions with child and adolescent mental health services (CAMHS). Many turned to the internet as source validation and to help them better understand and communicate their experiences in the absence of professional help. This was often evident amongst people with neurodiversity where waiting lists were reported to be up to 7 years.

I'm fortunate because my mum can afford to go private. But a lot of people can't and you know, they really need the help from the diagnosis. And you know, they just can't. And. Yeah. I just think there needs to be more support that isn't just CAMHS because it is so hard to access. (P020)

Autistic participants with a high level of need also highlighted the invalidation they experienced in educational settings. This highlighted that connection between mental health and autism was often confusing or poorly understood by participants, their immediate networks, and health and care professionals alike.

my teacher whilst I was in college said that it's a 'me' problem, why I'm like this, because he didn't understand my autism or no one did. ... And he directly said to me, a teacher, 'well basically all teenagers are like this, that's a 'you' problem'. (P09)

To avoid invalidation, and to feel able to relate to the people around them, many participants described different forms of masking. Masking showed up as drinking to avoid feeling different to others (e.g., to not feel autistic), code-switching (e.g., person changing their accent), not going out, hiding oneself/not being able to be authentic self (e.g., masking trans identity) and people pleasing. Masking contributed to a sense of anxiety and was sometimes described as losing their sense of self.

I sound like very different, so I have to code switch like I have to actively not use like my normal accent and it's very hard to like keep it up for like a long while. (P40)

Challenges in naming, articulating or processing emotions often led to rumination, or feeling stuck. Many participants reflected how their sense of self and awareness of mental health had evolved overtime as they had matured.

I feel like when you first start becoming a teenager, it's very like - it's quite a turning point. And you sort of become like, more aware of yourself and like, how you feel and things like that. So, it sort of started around the age of 12/13 when I became aware of it. Erm, I was quite confus- I was quite confused because I was struggling a lot with my emotions and how I reacted to things. Basically, my emotions were quite dysregulated. I had a hard time regulating them myself, so when I get really, really angry, it would be pretty much un- like, unstoppable or uncontrollable anger. (P079)

Some participants recognised it was important to focus on what was important to them rather than seeking validation from others or worrying about what others thought. This helped to increase confidence and positive feelings toward oneself.

And it's just the realisation that I needed to not be like everyone else because I'm not going to be like everyone else. I have autism and I'm different. Like this as much as you can be different like as much as you can be the same, you're not going to be different like. How I say to my mom how I describe it to everyone is I can do everything, but I do it the P164-way. I do it my own way. (P164)

Participants shared that it was important to separate beliefs relating to culture and religion. Poor mental health was often viewed as a weakness and something not to be spoken about in particular communities which often led participants to feel dismissed and their experiences invalidated. Several participants talked about mental health and help-seeking being viewed differently in England/UK compared to their cultures or countries of origin.

I think in India we don't really talk about, first of all, mental health is a taboo. If you have any mental health issues, it means you're weak, you're broken, you're a bit useless. So and in terms of like having support from university, that's not exactly a concept that exists in India. It's just like if you can't handle the pressure of this, then maybe you're not supposed to be here. But I was quite impressed with the way my university supported me. (P094)

3. Proactively coping and preventive opportunities

The intersections between mental health symptoms, triggers and behaviours remain complex. Anxiety and symptoms related to it dominated and the substantial impact on participant's lives was discussed, including to relationships, education and employment. Experiences of professional interventions, including medication and various therapies was mixed. Generally, more person centred and bespoke interventions generated better outcomes.

It affects it quite badly because I can't do what I used to do ... I used to work which I can't do at the moment. I stopped about ... about four years. ... It's affected my life doing things as well. Like going out - going out on my own quite regularly. When I hear voices, I can't go out on my own. (2P02)

Participants shared a wide variety of coping strategies that they employed whilst waiting for more formalised help. This demonstrated how proactive young people are as opposed to passively waiting on a list to be seen. These included relational, environmental and practical tools (such as games, fidget toys and apps), cognitive strategies, problem solving, holistic spiritual or religious practices, connection to animals, sensory, exercise and creative and arts based practices.

I really like swimming. That just the feeling of the water. I bizzarely like fizzy drinks, not I don't like things that are very very sweet, so it's sparkling water or, like a diet soda or like flavoured sparkling water. Very, very cold drinks or or like boiling hot coffee, depending on my mood. I have a weighted blanket and noise cancelling headphones. Wear a fidget ring. Got lots of lots of like, fidgety things at home. (P110)

Conscious coping mechanisms tending to be highly specific, including distraction actions, changing environment and sometimes simple endurance. Arts and culture are relevant to some with a bias towards doing (art therapy, writing, exercise). A focus on aspirations was prevalent as a positive force for mental health and some found their mental health experiences provided motivation to help others.

Yeah, so I chose Animal Care and I absolutely loved it. It really helped my mental health. ... Amazing, yeah. For some reason being attacked by geese is just calming.

Participants shared lots of opportunities where their story could have had a different outcome. Such examples often acted as an anchor from which participants shared their narrative. Within acknowledging the impact of trauma and adverse experiences, it was important to not disregard the norm and to recognise that poor mental health can occur in the absence of an obvious event. Some participants who haven't experienced trauma shared they did not feel that their experiences of poor mental health were justified in the absence of a clear antecedent.

Transition was recognised as a challenging experience and a time when support could be proactively offered, and referred to in four ways: getting older, moving home, changing educational institutions, and transition between mental health services and/or professionals.

So growing up, I lived in South Africa for my first couple of years of my life, and then I moved to the UK with my parents. And I lived in a very like a predominantly white neighbourhood. And that's where I went to school and I faced a lot of bullying in primary school, which wasn't very easy. And I would say that that's probably where, that was probably the underlying cause of a lot of things and the origin for a lot of stuff. (P097, ln32)

Whilst some participants reported accessing help through school, college or university, many shared that the education system was a source of stress and anxiety. The transition between educational systems was recognised as an area that more support could be provided particularly for neurodivergent participants, owing to the change in environment and routine.

I struggle a lot with transitions, and when I moved from my primary school to my secondary school, I found it very hard to make friends and things like that. So, I became, really very depressed. I struggled to function, I did go selectively mute with peers at school, (P165, ln70)

For autistic participants with a high level of need, secondary school was a standout theme as a substantially negative experience, characterised by a lack of flexibility towards autism, and anxiety in particular, and the insistence on a disciplinary model to counter all 'problems'. Experience of alternative provisions varied with the level of personalisation. Although relationships with family and friends are highly influential, they're specific and varied while, thematically, impacts that affect mental health tend to be negative. Life transitions are double-edged, heralding a sharp decline in mental health (e.g., at 11 to secondary), or new challenges and opportunities (e.g., college and employment).

And when I moved to secondary it was much harder. Because, um, they never helped with anything like dyslexia so I had really bad anxiety over it. And then I kind of stopped going to school for quite a while. ... Because I like struggled with like making friends but then also with noisy classrooms and um ... trying to think ... like actually like speaking to people and teachers and stuff I struggled with. So if like they picked me out in the classroom I'd be a bit like scared of that. (P07)

Being employed or having a voluntary position was often considered beneficial (owing to a sense of purpose and routine) despite the potential challenges of engaging in the workplace (such as demanding workloads or a lack of flexibility).

So I think in that way it was, it was quite healing to be able to do the job that I do where I get to express, you know, the experience that I had as a young person and shape the the places that didn't really help me at the time that I need them to help me. It's been quite a healing experience (P117, ln335)

Case studies highlighting the impact of intersectional identities

Owing to the biographical nature of data collection, it was felt important to highlight the intersectional experiences of those who were recruited. From discussions with the young people who worked on the data, the below case studies were felt to provide good examples of how intersecting characteristics impact mental health, and the participants ability to access support.

Case Study 1

The interview took place outside in a local park. P049 was very keen to take part and excited to learn more about the research '*I feel like I'm contributing to society*'. P049 struggled at first with the open nature of the interview, and started by sharing that he had only recently recognised the importance of mental health. He started by providing some general demographics, where he lived and where he went to school. When asked about his experiences at school he said that they '*support mental health and all that*'. He then shared that this related to a suicide attempt two years prior, and that the school had referred him to a social worker who he was no longer in contact with.

Sometimes it depends on. I guess your life, I felt like, but I feel like some people feel like they have so much responsibility on them to like. Follow. Follow what other people tell you to do, and this is like. You feel pressure to follow what other people tell you to do. That's like, I mean, maybe it's like sometimes that is made-up in your head, but sometimes it is true.

The suicide attempt was a result of feeling under pressure and an overwhelming sense of responsibility. For P049, this did not relate to school pressure or needing to choose a career, but to growing up in a household where he would be exposed to his Dad abusing his Mum. This led to struggling to process emotions and feeling unable to trust other people and form close relationships.

That's the only emotion I'm feeling ever in school when talk to other people, all I'm feeling is that one emotional, like I'm pissed.

P049 recognised that he had not coped well with his mental health, using distractions and smoking, but stated that he was '*trying to cut that*'. He shared that he had two sessions with a school counsellor and then quit but had social workers coming over every weekend after his suicide attempt. He found talking about his mental health painful, but insightful.

I think I think I already said this, but it was like. So painful for me to talk to them [school counsellor] because I was like I've never really talked. Even now I've not really talked too much about how I should like ever. So it's just like. When they're when you're talking to, I get. This is not even just mental health, I guess that's in general too. But like when I talk to someone else, sometimes you like, you realise things that you never realised before about how you feel.

P049 still lives with his Dad but hadn't spoken to him for 7 months. His parents are both Pakistani and Muslim and don't talk about emotions. P049 shared that he is still struggling with his mental health, but that he enjoys his volunteering and the positivity of the environment and interacting with different types of people. At the end of the interview he talked about the importance of not seeking validation from others, and asked to be sign posted to self-referral mental health services as the interview had given him confidence that he could talk about his mental health with people, whereas he had previously talked about not being able to 'get deep'.

Like a depressed suicidal kid as I have. If I talk to a lot of people. If I do this, and if I do a lot of people that normal people find normal, you know what I mean? So I'd say trying to validate trying to get validation from others trying to piece from others, but. One of the worst things you can do. Because it gets you thinking about. What do other people want? They should be thinking about what you want.

Case Study 2

The participant initially appeared nervous, and time was spent grounding in the space before beginning the interview. P152 felt that having parents from different ethnic backgrounds led to a sense of not fully belonging to either culture. She also described how cultural beliefs around mental health, alongside one parents' attitudes towards mental health, impacted her ability to seek help.

Well, I've been thinking about how in like the kind of cultural and ethnic background that I'm from, mental health isn't something that people really talk about or they talk about in a way that's like, if you've got mental health problems, then you must have done something wrong or you're not like, your faith isn't strong or something like that.

Her position as the eldest daughter in the family also led to difficulty feeling able to acknowledge or express difficulties with mental health.

Like as an eldest daughter, you're not really allowed to have problems because everyone else has problems. You're always looking after everyone else.

She described difficulties coping during secondary school, including the need to mask and experiencing meltdowns. During sixth form, she sought help from her GP but found it unhelpful due to a lack of understanding of her disordered eating being linked to a need for control rather than related to body image. She also experienced OCD and body dysmorphia, which led to repeated checking of her appearance and mask wearing, brought on by COVID. P152 also questioned her own experiences, often wondering whether she was exaggerating what she was going through. Currently, she describes anxiety as being most challenging but feels more able to process rather than internalise her anxiety. P152 began researching autism and self-identified as autistic before later receiving a diagnosis through Right to Choose. P152 faced challenges at university, including difficulties with making friends, but found the university supportive and accessed reasonable adjustments. She attended a peer support group for autistic people, where she found both the group and a peer mentor through the forum to be supportive and accepting. P152 described not feeling represented as a Muslim autistic person.

And I just, I feel, I want to know more people like me and I wish that like, sometimes I search online like just on Google, and I want to see like what Muslim autistic people there are. And the autistic women, and there are some, but there's not much about them.

P152 used a range of coping strategies, including wearing earplugs to manage noise and using fidget toys. She also watched videos to learn coping strategies, wrote to explore and make sense of friendships, and prayed.

She identifies as asexual and feels that this is not well understood or represented within her cultural background or the media. She also described religious expectations around marriage as being challenging.

P152 found the narrative style of the interview unusual and reflected that she had not previously had the opportunity to express these thoughts aloud

Case Study 3

The interview took place in a community hub local to the participant. She came alone. 2P07 appeared nervous at first and her initial statement was thought through carefully as she talked with gaps. She struggled with anxiety and dyslexia at primary school.

I struggled a lot in primary school with people because they - I knew there was something, like, wrong because I wouldn't - I'd refuse to even go into school if like the sun was too bright, because it hurt my eyes and it was just very overwhelming. And I was always like different from my siblings. ... and I'm just surprised no one really noticed.

2P07 has a non-identical twin sister who doesn't seem to have the same issues but they were kept together for special classes etc, which was eventually problematic for the sister. Parents didn't engage with issues early on leading 2P07 to 'mask' and try to be more like her sister. Parents split up during primary school (8), moving back and forth added to stress.

And my parents kind of pushed it to the side. They didn't really want to, um, want to get like any diagnosis or see what was going on which ended up like kind of me trying to like almost mask myself and be more like my twin sister ... but it just never worked

The move to secondary school was hard. 2P07 struggled with noisy classrooms (wasn't allowed to wear headphones), and stopped going to school for a while. 2P07 struggled with making friends, anxiety, tics (diagnosed with Tourette's), light sensitivity. The anxiety leads to stress, hands shaking, and migraines. 2P07 felt different growing up, more imaginative and weird than her peers. In Y8 (12) she was put in a separate classroom just after Covid, which as described sounds like a neurodivergent break out space. It was a difficult experience with 2P07 feeling isolated and with nothing to do (no wif-fi), and she lost friends. Covid was a watershed for anxiety: having got used to isolating, the change back was traumatic. Shortly after, 2P07 took an overdose and was referred to CAMHS – Tourette's had become serious and was also a referral factor. 2P07 didn't like the person working with her at CAMHS and the sessions were not helping at all. The psychologist just repeated coping mechanisms and counsel even after feedback that it wasn't working. 2 years later, 2P07 was referred to MARS, a joint health and social care crisis response service for children and young people with acute mental health difficulties in Cornwall who don't need an admission. She found the team 'quite nice'. She also had a youth worker who 2P07 didn't get along with. Currently 2P07 is back with CAMHS but it's better because she has art therapy there. CAMHS have recommended an ASD diagnosis be done but it hasn't happened yet. Sleep medication was prescribed by CAMHS but 2P07 has been sleeping badly since childhood. Anti-depressants prescribed by MARS have made a dramatic improvement to anxiety and hence her life. 2P07 hopes a diagnosis will mean people take her concerns seriously.

in like my school, they didn't like take anything seriously ... And then now when I went into college ... I have neurodivergency put in my records. I'm put in. I could leave my classroom whenever I want, go for a break. I can wear my headphones, I can do basically whatever I need to keep myself like OK in college. So I'm much - it's much nicer in college. ... It could have been because I'm around different people, - it was how the people saw it, but it's like made it much easier.

2P07 found the narrative interview unusual as she feels she's not good at talking about herself and usually needs prompts.

Discussion [797 words]

This study gathered experiences concerning mental health from diverse young people who felt under represented and whose voices are rarely present in existing literature. 81 young people shared their stories, providing a holistic view of their life course and experiences. Many participants were motivated to take part by the possibility of helping others, and it was the first time sharing their story in its entirety without fragmenting it. Young people shared challenges in identifying their experiences as poor mental health, this was often facilitated by relating to others experiences. When seeking help for poor mental health many participants reported feeling invalidated, and encountering stigma, as well as systemic challenges such as not being 'poorly enough' to access services. However, young people were very proactive and resourceful in finding ways to cope with poor mental health when they could not access formal services. Many young people reported forms of adverse childhood experiences such as, racism, bullying, domestic abuse and bereavement, and times of transition were identified an important preventive opportunity.

This research highlights the importance of recognising and addressing poor mental health in young people in a timely way. The impact of common mental disorders such as anxiety, and experiences relating to neurodivergence are often minimised as being less severe, and thus not warranting attention or resources[21, 22]. However these experiences can have substantial long term impacts, often affecting young people's ability to complete education and obtain subsequent employment[23].

The narratives shared in this research support existing literature that highlights the complexity and prevalence of poor mental health amongst 16 – 25 year olds [2, 24]. In particular the need for better evidence informed and accessible interventions, that are readily available at times of transition such as starting school or university was highlighted. Support provided by existing health and education systems was challenging to access independently of someone to facilitate navigating the system that often relied on a diagnosis, a peer, parent or school counsellor. Participants often shared that they were stuck on waiting lists and when support had been accessed it was not always experienced as beneficial. A retrospective cohort study across England found that outcomes following routinely delivered psychological treatments were poorer in young adults than working age adults[25]. This highlights the need for support that is tailored to this age group (16 – 25 years old). In this research better outcomes often related to personalisation at the first point of contact, and timely person centred care. A systematic review concerning the implementation of person centred care amongst children and young people, highlighted the need for improvements in the provision of person centred care in this population and better training to support professionals to work flexibly to meet the needs of young people[26].

Despite the success of recruiting a large sample for a qualitative study, there were many challenges encountered during recruitment. On NHS waitlists contact details were often held for parents rather than young people themselves, and it was difficult to identify waitlists where young people would be eligible owing to the differences in how primary and secondary care was commissioned in different NHS systems across the country. Community recruitment was more successful, but often relied on existing relationships with individual organisations.

This body of evidence has implications for both policy and clinical practice. The young people who contributed to this research rarely felt heard or represented and benefitted greatly from being listened to and having their experiences validated. In the current health systems there is little capacity or space that allows for young people to reflect and share their experiences in this way, or provide feedback on whether they are finding the support provided helpful.

With regard to policy, school culture and recognition of adverse childhood events often impacted the onset of poor mental health. Better systems that both recognise and facilitate access to effective interventions are needed. In the development of such systems it is important that the experiences of people who are from minoritised ethnic groups and-or neurodivergent are considered to prevent exacerbating existing inequalities.

Conclusion

This body of evidence highlights the breadth and complexity of intersectional identities that contribute to poor mental health amongst young people aged 16-25. Narrative inquiry methods enabled young people to tell their stories in a way that felt important to them, and foregrounded a sense of agency and validation. There is a need for better systems to not only recognise mental health challenges in young people (via education and health systems), but also to provide safe spaces for young people to explore concepts around mental health and develop their own health literacy. The support provided for young people needs to be accessible but also flexible to allow for young people to provide feedback around what works for them and what does not, to facilitate more person centred care.

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Author contributions

RM – Drafted, revised and finalised the manuscript, wrote the ethics application (including the protocol) and facilitated the process of gaining relevant approvals, provided training to the young people and led data collection and analysis, chaired weekly project meetings, facilitated reflexive spaces throughout these processes drawn from existing NIHR funded work on inequalities and intersectionality (Co-PACT, Co-PICS and Oxford Health BRC), conducted the majority of in person interviews.

RF – Implemented and obtained ethics amendments relating to the cohort recruited in Cornwall, completed all data collection in Cornwall, led the analysis for the cohort recruited in Cornwall.

LC – Co-Applicant on the funding application and both led and supported all young people involvement, attended interviews as a peer researcher, and contributed to the analysis of all participants (including the Cornwall cohort).

LN -Facilitated recruiting peer researchers and the young people advisory group, as well as getting feedback from the young people on the ethics documents, conducted the screening calls with potential participants and the majority of the online interviews, contributed to the initial coding framework for the analysis.

NB – assisted the analysis of the transcripts, and summarising the data

FM (OM) – supported the young people working on the programme, assisted data analysis by listening to transcripts to verify data, coding transcripts and contributing to discussions around themes.

AN – attended interviews as a peer researcher, assisted data analysis by listening to transcripts to verify data, coding transcripts and contributing to discussions around themes.

RMa – attended interviews as a peer researcher, assisted data analysis by listening to transcripts to verify data, coding transcripts and contributing to discussions around themes.

RC - assisted data analysis by listening to transcripts to verify data, coding transcripts and contributing to discussions around themes.

LA - supported the young people working on the programme, attended interviews as a peer researcher,

HA – Co-applicant on the funding application to support dissemination and relationship with cultural partners, assisted with all recruitment.

RSS – Led the funding application and the pilot work that preceded the funding application, had the idea, wrote the published protocol, had oversight of the team and youth involvement, attended weekly project meetings and training sessions for the young people.

RSha – Co-applicant on the funding application and co-led the work package that this data relates to

JA – Co-applicant on the funding application and co-led the work package that this data relates to, led training to the young people on the methodology.

KB – Programme leadership/management, supervision, methods expertise in mixed methods, especially qualitative, narrative, ethnographic, biographical inquiry and data analyses, drawn from other adolescent and adult health programmes (ATTUNE, CREATE, Co-Pact, Co-Pics, Insights for UKRI and NIHR HSDR and NIHR BRC funded). Expertise in cultural, ethnicity, intersectional and health inequalities research practice and methods to inform policy. Writing the funding application for WP1/2 with JA, RS.

Conflicts of interest

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